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A. Purpose

To provide guideline instructions for General Nursing care with the aims that

- Needs and expectations of patients are established,
- Patient satisfaction is enhanced on continual basis.

B. Scope

It covers all in patients receiving treatment in the hospital.

C. Responsibility

Nursing In-charge and Ward nurse.

D. General Instructions for Nurses

1. Discipline

DO's

- Attend duty in proper uniform within 10 minutes before reporting time
- Always display Identity Card.
- Take over inventory articles by counting each item.
- Take over each patient from previous staff with minute details.

DON'TS

- Come late for duty
- Use nail polish, have long nails or wear extra ornaments.
- Accept gifts/ money from patients/attendants.
- Use cell phones/mobile phones on duty.

2. Orientation to patients and their attendants at the time of admission.

DO's

- Address the patients/relatives courteously.
- Accompany the patient to room
- Offer the bed to patient and make him/her feel comfortable
- Make sure nurses are always available at the time of call.
- Inform the patients about
  - Visiting hours and regulations
  - Timings of hot water availability
  - Timings of air conditioning (cold & hot)
  - Location of the pharmacy to get medications.
  - Location of drinking water
  - Provision of water supply
  - Food timings.
  - Availability of barber in day time.

- Remove ornaments & valuables in the presence of a relative
- Note down the name and number of each item on the back **of in the initial assessment form.**
- Get the name, signature and the relationship of the receiver.
- Record activities like sponge bath, back care, mouth care, after giving the needy patients.
- Inform the patient well in advance about various procedures like operation/investigations etc.

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- Make sure the patient is seen by the doctor soon after admission and whenever there is a complaint.
- Informs the medical officer –in-charge on the spot in case the doctor on duty doesn't respond to the call.

3. Courtesy

DO's

- Be polite and courteous to the patients/attendants/visitors.
- Attend to the patient on one call.

DON'Ts

- Talk improperly/ rudely to the patients/attendants
- Argue with patients/ attendants/ Visitors/ Co-workers/ Supervisors
- Ignore any problem mentioned by the patient/ attendants
- Allow smoking or drinking in the premises
- Allow eatables and flowers into the hospital.
- Engage in lengthy talks over telephone.

4. Procedures /Investigations

DO's

- Send patients to other departments or for investigations with Ward attendants only.
- Start blood transfusion with the knowledge of patient/relative only.
- Switch off electrical appliances when not in use (geysers, needle destroyers, hot plates, fans, refrigerators etc.)
- Always lock vacant rooms after getting thoroughly cleaned.

DONT's

- Allow valuables and money with the patient
- Allow attendants to sit or sleep on patient's bed
- Allow the patient to leave hospital without written permission from the doctor
- Leave the medical record in patient's room/bed
- Allow the patient/attendants to carry the medical record to any place.
- Filling of investigation forms by nursing staff
- Send the patients without rails or cover sheets/blankets
- Allow the Ward attendant & Sanitary Attendants to leave the ward/department without information.
- Allow the Ward attendant & Sanitary Attendants to leave the ward/ department before handing over the responsibility to next shift person.

5. Medicines & I.V. Fluids

DO's

- Start medications immediately after admission/orders
- Remember 5 "R" before administering any medication
  - Right Drugs
  - Right Dose
  - Right Route
  - Right Time
  - Right Patient

- Make sure the patient takes the medicine in your presence
- Check blood sugar before the food reaches the patient.
- Give insulin injection before relative/ patient reminds you
- Make a loop and fix the I.V. tube – prevent cannula from coming out.
- Put date to I.V. Cannula site, I.V. tubings, urinary catheters, Ryles Tubes etc.
- Regular care of I.V. sites always (after removing cannula also)
- Procure medicines speedily from pharmacy
- Check the balance of medicines before indenting

- Inform Nursing Matron /Consultant –in-charge on the spot about medicines not available.
- Check portable O2 Cylinders in each shift, note down reading in register.
- Ensure enough water in O2 humidifiers.

DON'T's

- Record before giving medicine/injections.

6. Infection Control/ Waste Management

DO's

- Hand wash in between procedures to avoid spread of infection.
- Ensure waste segregation, burning of needles, cutting the tips of syringes, cutting tubing after each use and keep them in 1:9 Sodium hypochlorite solutions.
- Change Sodium hypochlorite solution in every shift
- Use colour coded bags after proper segregation of the waste
- Keep infected waste in yellow bag only
- Ensure cleaning of suction jars after every use.
- Ensure barrier nursing universal precautions to infectious patients.

- Ensure carbolising the beds and the unit after a patient is discharged to keep the room ready for next patient.

DONT's

- Wear ICCU/ Cath Lab/ OT dress/ slippers out of the department
- Keep infected dressing materials in dressing trolley
- Keep sterile gauze drum for more than 24 hours.
- Keep the CSSD items for more than 48 hours
- Prick more than once during venepuncture
- Touch the area of I.V. site after cleaning and before inserting the cannula.

7. Discharge Process

DO's

- Remove I.V. cannula at the time of discharge
- Send the file with discharge summary to billing office with the ward attendant accompanied by the relative of the patient.
- Check the payment receipt number from accounts section before releasing patients (for paying patients only).

- Hand over the discharge summary, master chart to patients/relatives with instructions.
- Send the patient on wheel chair up to the entrance with the Ward attendant.

DONT's

- Take more than 3 hours for discharge process.

NURSING PROTOCOL

E. ORAL CARE

1. **Definition**

Care of the Oral including teeth, gums, lips and cheeks.

2. **Purpose**
   - To give a feeling of freshness.
   - To prevent infection.
   - To give a sense of well-being.
   - For aesthetic sense.
   - To maintain cleanliness.
   - To prevent bad odour.
   - To stimulate appetite.

3. **Articles Required**

- A tray containing
  - Small mackintosh and towel
  - Face towel
  - Small jugs – 2 (one with hot water, one with cold water)
  - Feeding cup/glass
  - Artery forceps – 1
  - Dissecting forceps – 1
  - Gauze pieces (in a bowl)
  - Dentifrice (in container)
  - Solution for mouth wash
  - Emollient
  - Swab sticks in a bottle
  - Mouth gag (in case of unconscious patient)
  - Tongue depressor (in case of unconscious patient)
  - A bowl with clean water
  - Kidney tray and paper bag
- Screen for privacy

4. Procedure 1 (For the patient who is able to care for himself)

- Explain the procedure to the patient.
- Provide privacy
- Give a comfortable position to the patient (sitting or Fowler's position with cardiac table in front or lateral with face at the edge of pillow)
- Place the mackintosh and face towel across the chest (if patient is sitting / on the pillow if patient is in lateral position)
- Place kidney tray close to the cheek
- Remove dentures and place in a bowl of clean water
- Arrange the articles.
- Wash hands.
- Prepare the mouth wash by mixing hot and cold water and one crystal of KMnO4
- Help the patient to rinse his mouth.
- Let the patient hold kidney tray as per his convenience for return flow.
- Pick up the tooth brush, wet it with water, spread tooth paste on it and hand it over to the patient.
- Instruct the patient to brush all sides of the teeth extending from the gum to the enamel.
- Pour water on brush, holding it over kidney tray and clean brush thoroughly and put back the brush.
- Help the patient to rinse his mouth thoroughly.
- Help the patient to wash his face and hands. Wipe with the towel.

5. Procedure 2 (For an unconscious patient)

- Explain procedure to patient's attendant (if present)
- Provide privacy
- Place patient in lateral position with face at edge at end of pillow
- Place the mackintosh and face towel on the pillow
- Arrange the articles.
- Wash hands.
- Prepare the mouth wash by mixing hot and cold water and one crystal of KMnO4
- Make a paste with Soda bicarbonate or salt or any available dentifrice.
- Place the kidney tray close to the cheek.
- Take a gauze piece; wrap it around artery forceps, covering the tips completely.
- Moisten the gauze and dip it in the cleansing agent. Swab each tooth gently but firmly cleaning all sides of the teeth. The used gauze can be removed from artery forceps with the help of a dissecting forceps.

- To clean the inner and chewing surface of teeth, use a mouth gag to help the mouth open.
- With mouth gag in position, clean the tongue using the gauze covered artery forceps.
- Wipe the face with towel.
- Apply Boro-glycerine on lips and tongue with swab sticks.
- Remove the kidney tray, mackintosh and towel.
- Make the patient comfortable, tidy up the unit.
- Discard the wastes and clean the articles with soap and water. Boil the forceps.
- Wash hands.
- Record the time, solution used for mouth cleaning and condition of the mouth on the nurse's record.

Note:

Common dentifrices

- Tooth powder
- Glycerin with lime juice
- Neem stick
- Sodium bicarbonate paste

Common Mouthwash Solutions

- Potassium permanganate (1:5000)
- Hydrogen peroxide (1:8)
- Normal saline
- Sodium chloride (1tablespoon to a pint of water)

Common emollients

- Boro-glycerine
- Olive oil
- White Vaseline
- Body Cream
- Liquid paraffin

NURSING PROTOCOL : CHECKLIST FOR ORAL CARE

Patient's Name *:* _____

MRD No *:* _____

Ward/Unit *:* _____

Sr. No.	Checklist	Yes	No	Remarks
	Procedure I: (For the patient who is able to care for himself)			
1.	Explain the procedure to the patient.			
2.	Provide privacy.			
3.	Give a comfortable position to the patient (sitting or Fowler's with cardiac table in front or lateral with face at the edge of pillow).			
4.	Place the mackintosh and face towel across the chest (if patient is sitting / on the pillow if patient is in lateral position).			
5.	Place kidney tray close to the cheek.			
6.	Remove dentures and place in a bowl of clean water.			
7.	Arrange the articles.			
8.	Wash hands.			
9.	Prepare the mouth wash by mixing hot and cold water and one crystal of KMnO4.			
10.	Help the patient to rinse his mouth.			
11.	Let the patient hold kidney tray as per his convenience for return flow.			
12.	Pick up the tooth brush, wet it with water, spread tooth paste on it and hand it over to the patient.			
13.	Instruct the patient to brush all sides of the teeth extending from the gum to the enamel.			
14.	Pour water on brush, holding it over kidney tray and clean brush thoroughly and put back the brush.			
15.	Help the patient to rinse his mouth thoroughly.			
16.	Help the patient to wash his face and hands. Wipe with the towel.			

	Procedure II: (For an unconscious patient)			
1.	Explain the procedure to the patient.			
2.	Provide privacy.			
3.	Position the patient			
4.	Place the mackintosh and face towel across the chest (on the pillow if patient is in lateral position).			
6.	Arrange the articles.			
7.	Wash hands.			
8.	Prepare the mouth wash by mixing hot and cold water and one crystal of KMnO4.			
9.	Make a paste with Soda bicarbonate or salt or any available dentifrice.			
10.	Place the kidney tray close to the cheek.			
11.	Take a gauze piece; wrap it around artery forceps, covering the tips completely.			
12.	Moisten the gauze and dip it in the cleansing agent. Swab each tooth gently but firmly cleaning all sides of the teeth. The used gauze can be removed from artery forceps with the help of a dissecting forceps and discard in paper bag.			
13.	To clean the inner and chewing surface of teeth, use a mouth gag to help the mouth open.			
14.	With mouth gag in position, clean the tongue using the gauze covered artery forceps.			
15.	Wipe the face with towel.			
16.	Apply Boro-glycerine on lips and tongue with swab sticks.			
17.	Remove the kidney tray, mackintosh and towel.			
18.	Make the patient comfortable, tidy up the unit.			
19.	Discard waste and clean articles with soap and water.			
20.	Wash hands.			
	Record the time, solution used for mouth wash and condition of the mouth on the nurse's record.			

Name:

Signature:

Date:

F.BED / SPONGE BATH

1. Definition

Sponge bath is defined as bathing a patient who is confined to bed and who does not have the physical and mental capability of self bathing.

2. Purpose

- To make the patient comfortable and fresh.
- To improve circulation.
- To observe skin for redness, injuries, swelling, rashes or other infections and bony prominences for bed sores.
- To prevent pressure sores.

3. Articles Required

- A screen, bath blanket for privacy or top sheet.
- Wash / Sponge cloths – 2
- Bath towel – 2
- Face towel – 1
- A new set of clothing
- Contains of hot and cold water.
- Bath basin.
- A tray containing soap, methylated spirit, talcum powder, hair oil, comb, nail cutter, kidney tray, paper bag, duster to clean to locker.
- Laundry bag.

4. Procedure

- Assemble all the articles at bedsides.
- Patient is to be asked to pass urine before beginning the procedure.
- Explain the procedure to the patient, if conscious.
- Screen the patient.
- Cover the patient with top sheet. Fanfold other top clothes at foot end.
- Mix cold and hot water in basin from containers and check the temperature on the back of your hand. (Already mixed warm water can also be brought in a basin).
- Remove clothing and put in laundry bag.
- Place the towel on chest of the patient or under the head.
- Fold the sponge cloth in your hand to make a mitten, wet it, and apply soap on it and clean face, neck, behind the ears thoroughly.
- Rinse, sponge cloth and clean face with fresh water and dry face, neck etc. with face towel.

- Expose farther hand, spread bath towel underneath, fold sponge cloth around hand, wet, apply soap and clean hand, axilla, thoroughly using long, firm strokes from distal to proximal end.
- Wipe with plain water and dry. (Patient's hands can be immersed in basin for 3-5 minutes).
- Repeat the same on other arm.
- Expose patient upto waist, spread towel on chest, clean chest thoroughly with soap and then water from under the bath towel dry and cover again with top sheet.
- Expose patient upto pubic region, place bath towel over chest and abdomen, wash, rinse and dry abdomen lifting edge of the bath towel, in side to side strokes.
- Turn the patient towards your side. Spread bath towel under back, expose back and clean with soap and fresh water in circular motion and dry with bath towel.
- Massage back with a spirit in circular motion, apply talcum powder.
- Expose farther leg, spread bath towel under the leg, flex the knee clean with soap and water using long, firm strokes, dry and cover (foot can be allowed to soak while you wash leg).
- Do the same on other leg.
- Clean private parts in same way or ask patient to do it himself/ herself.
- Put on clean clothes, comb hair, cut nails.
- Remove bath blanket, cover with top cloths.
- Remove and replace articles.
- Leave patient comfortable and unit tidy.
- Record if any abnormal changes have been observed in skin and bring it to the notice of ward sister.

NURSING PROTOCOL: CHECKLIST FOR BED / SPONGE BATH

Patient's Name : _____

I.P.No. : _____

Ward : _____

S. No.	Checklist	Yes	No	Remarks
1.	Assemble all the articles at bedside.			
2.	Patient should empty bowel and bladder before beginning the procedure.			
3.	Explain the procedure to the patient, if conscious			
4.	Screen the patient.			
5.	Cover the patient with top sheet. Fanfold other top clothes at foot end.			
6.	Mix cold and hot water in basin from containers and check the temperature on the back of your hand. (Already mixed warm water can also be brought in a basin).			
7.	Remove clothing and put in laundry bag.			
8.	Place the towel on chest of the patient or under the head.			
9.	Fold the sponge cloth in your hand to make a mitten, wet it, and apply soap on it and clean face, neck, behind the ears thoroughly.			
10.	Rinse, sponge cloth and clean face with fresh water and dry face, neck etc. with face towel.			
11.	Expose farther arm, spread bath towel underneath, fold sponge cloth around hand, wet, apply soap and clean hand, axilla, thoroughly using long, firm stokes from distal to proximal end.			
12.	Wipe with clean with water and dry. (Patient's hands can be immersed in basin for 3-5 minutes).			
13.	Repeat the same on other arm.			
14.	Expose patient upto waist, spread towel on chest, clean chest thoroughly with soap			

	and then water from under the bath towel dry and cover again with top sheet.			
15.	Expose patient upto pubic region, place bath towel over chest and abdomen, wash, rinse and dry abdomen lifting edge of the bath towel, in side to side strokes.			
16.	Turn the patient towards your side. Spread bath towel under back, expose back and clean with soap and fresh water in circular motion and dry with bath towel.			
17.	Massage back with a spirit in circular motion, apply talcum powder.			
18.	Expose farther leg, spread bath towel under the leg, flex the knee clean with soap and water using long, firm strokes, dry and cover (foot can be allowed to soak while you wash leg).			
19.	Do the same on other leg.			
20.	Clean the private part in same way or ask patient to do it herself.			
21.	Put on clean clothes, comb hair, cut nails.			
22.	Remove bath blanket, cover with top clothes.			
23.	Remove and replace articles.			
24.	Leave patient comfortable and unit tidy.			
25.	Record if any abnormal changes have been observed in skin and bring it to the notice of ward sister.			

Name:

Signature:

Date:

G. Back care

1. Objectives of Purposes

Back care is given to -

- Cleanse the skin
- Promote circulation by stimulating the skin's nerve endings and underlying tissues.
- Help relax a restless person.
- Help to prevent bedsore and keep the skin intact.
- Dilate superficial arterioles, which bring more blood and nourishment to the skin.
- Eliminate waste products from the body through the skin.
- Promote comfort through muscle relaxation and skin stimulation.
- Give the nurse an excellent opportunity to strengthen the nurse-patient relationship.

2. General Instructions:

- Check the physician's orders to see the specific precautions if any, regarding positioning and movement of patient.
- Assess the patient's need for back care.
- Assess the patient's mental state to cooperate and to follow directions.
- Assess the cardiac respiratory functioning. Check T.P.R. and B.P.
- Check the patient's preference for soap, powder etc.

3. Preparation of articles

- Bath basin - 1
- Small bowl - 1
- Soap with soap dish
- Wash clothes - 2
- Bath towels - 2
- Bath blanket on sheet.
- Methylated spirit and powder.
- Kidney tray and paper bag.
- Jug - 2
- Bucket - 1
- Clean linen
- Laundry bag

4. Preparation of patient and unit

- Explain the procedure to the patient.
- Remove unnecessary items from the work area and place the articles needed conveniently on the bedside table.
- Bring the patient to the edge of the bed and towards the nurse to prevent overreaching.
- Check the room temperature and warm it if necessary.
- Close the windows if necessary and put off the fan to prevent draughts.
- Provide privacy by means of curtains.
- Remove the top bed linen on fan – fold them to the foot end of the bed, leaving a sheet or bath blanket over the patient. Keep it free at the foot end to allow freedom for the legs.
- Keep patient in prone or side lying patient according to the patient's condition.
- Remove the personal clothing and cover the patient with the bath blanket.

5. Procedure

- Wash hands.
- Mix hot and cold water in the basin and check the temperature on the back of the hand. Fill the basin half full.
- Assist the patient to turn to a prone or side-lying position. Position the bath blanket and towel to expose only the back till buttocks with the face away from the nurse. Make sure that the patient will not fall.
- Fold back the bath blanket from the shoulder to the thighs and tuck the edges secularly around the thighs. Place the towel over the bed, close to the back, lengthwise.
- Wash, rinse and dry the patient's back from the shoulders to the buttocks with brisk circular movements.
- After drying the back give a though back rub in circular motion with methylated spirit and powder.
- Pay particular attention to the pressure points and cleansing between gluteal folds.
- And observe for any indication of redness or skin breakdown in the sacral area.
- Put on the clean gown / patient clothes.
- Wash hands.
- Record the observations on the Nurse's Daily Record with date and time.

6. **After care of the patient and articles**
   - Replace the patient's personal clothing.
   - Straighten the bed linen.
   - Remove the bath blanket and put it for washing.
   - Change the bed linen if needed.
   - Position the patient for comfort and proper alignment.
   - Take all articles to the utility room. Disinfect the bath basin and the wash clothes. Send the soled linen to the laundry.
   - Put back all the articles in the proper places after cleaning. Personal articles are replaced into the bedside table.
   - Take the opportunity to teach the patient on his relatives about the personal hygiene
   - Wash hands
   - Record the observation in the nurse's daily notes with date, time, and condition of skin like redness, breaking skin, etc.

NURSING PROTOCOL : CHECKLIST FOR BACK CARE

Patient's Name:_____

I.P. No._____

Ward/Unit: _____

S. No.	Check List	Yes	No	Remarks
1.	**Preparation of patient and unit:** Explain the procedure to the patient.			
2.	Remove unnecessary items from the work area and place the articles needed conveniently on the bedside table.			
3.	Bring the patient to the edge of the bed and towards the nurse to prevent overreaching.			
4	Check the room temperature and warm it if necessary.			
5	Close the windows if necessary and put off the fan to prevent draughts.			
6	Provide privacy by means of curtains.			
7	Remove the top bed linen or fan – fold them to the foot end of the bed, leaving a sheet or bath blanket over the patient. Keep it free at the foot end to allow freedom for the legs.			
8	Keep patient prone or on side according to the patient's condition.			

9	Remove the personal clothing and cover the patient with the bath blanket.			
1	**Procedure:** Wash hands.			
2	Mix hot and cold water in the basin and check the temperature on the back of the hand. Fill the basin half full.			
3	Assist the patient to turn to a prone or side-lying position. Position the bath blanket and towel to expose only the back till buttocks with the face away from the nurse. Make sure that the patient will not fall.			
4	Fold back the bath blanket from the shoulder to the thighs and tuck the edges secularly around the thighs. Place the towel over the bed, close to the back, lengthwise.			
5	Wash, rinse and dry the patient's back from the shoulders to the buttocks with brisk circular movements.			
6	After drying the back give a thorough back rub with methylated spirit and powder.			
7	Pay particular attention to the pressure points and cleansing between gluteal folds.			
8	Observe for any indication of redness or skin breakdown in the sacral area and bony prominences.			
9	Put on the clean gown / patient clothes.			
10	Wash hands.			
11	Record the observations on the Nurse's Daily Record with date and time.			
1	**After care of the patient and articles:** Replace the patient's personal clothing.			
2	Straighten the bed linen.			
3	Remove the bath blanket and put it for washing.			
4	Change the bed linen if needed.			
5	Position the patient for comfort and proper alignment.			
6	Take all articles to the utility room. Disinfect the bath basin and the wash clothes. Send the soled linen to the laundry.			
7	Put back all the articles in the proper places after cleaning. Personal articles are replaced into the bedside table.			
8	Take the opportunity to teach the			

	patient on his relatives about the personal hygiene.			

Name:

Signature:

Date:

H. ORAL MEDICATION

1. Definition

To administer drugs by oral route (mouth).

2. Purposes

- For therapeutic & symptomatic treatment of diseases.
- For prophylactic measures.
- For building up general health & supplementing diet.

3. General Instructions

- Always give medicines from a written order signed by doctor.
- Medicine bottle should be clearly labeled. Never give medicine from unlabelled bottles. Never give expired medicines.
- Don't keep labeled poison drugs on the medicine trolley.
- Never give medicine on empty stomach unless ordered.
- Medications should not be mixed with large quantities of food (e.g. milk).
- Keep 5 R's in mind, i.e. right patient, right time, right drug, right dose & right route.
- The nurse should know about each medication. She is administering, common usage & dosages) contraindications, side effects & toxic effects.
- Always check the label of medicine thrice before taking out from the bottle i.e. before pouring out in the glass, before administering and before replacing the bottle.
- Pour liquid medicine away from label to avoid spilling on the label.
- Never give the medicine, which is prepared by others.
- Record the time of the medicine given & put the signature.
- Ensure that patient takes medicine in front of you before you leave him.
- Don't handle tablets, pills & capsules with bare hands.

- If any time, one dose is missed by the patient due to some investigation etc., don't give the missed dose without consulting the doctor.

4. Articles Required

- A tray containing
  - ounce glass
  - water in a glass
  - containers for keeping tablets
  - medicine cards
  - paper bag
  - Small towel/ big incase of children, droppers/syringe.

5. Procedures

- Wash hands
- Set up the tray.
- Check the orders twice in the medicine book .
- Go and take out the required medicines after checking the medicine card & rechecking the label of the container, dosage, expiry date once again before you keep back the bottle in place.
- For liquids, shake the bottle well before pouring.
  - Remove the lid, hold the medicine in the right hand & check the label again.
  - Hold the medicine glass in your left hand at eye level.
  - Pour the exact amount of medicine in the medicine glass, keeping away from the label.
  - Wipe the mouth of the bottle, check the label once again & replace the bottle.
- Carry the tray with medicine card to the patient.
- Call the patient by name to check that you are giving medicines to the right patient; explain the procedure to the patient.
- Give the medicine to the patient one by one with water.
- Record the time, dosage, and medicine on the nurse's notes with signature.
- Keep all the articles after washing (glasses, containers etc.) dry & replace.

NURSING PROTOCOL : CHECKLIST FOR ORAL MEDICATION

Patient'sName : _____

I.P.No. : _____

Ward/Unit : _____

S. No.	Check List	Yes	No	Remarks
1.	Washes hands			
2.	Sets up the tray.			
3.	Checks the orders twice in the medicine book & write down the order in the medicine card.			
4.	Takes out the required medicines after checking the medicine card & rechecking the label of the container, dosage, expiry date once again before keeping back the bottle in place.			
5.	For liquids, shakes the bottle well before pouring. - Removes the lid, holds the medicine in the right hand & checks the label again. - Holds the medicine glass in left hand at eye level. - Pours the exact amount of medicine in the medicine glass, keeping away from the label. - Wipes the mouth of the bottle, checks the label once again & replaces the bottle.			
6.	Carries the tray with medicine card to the patient.			
7.	Calls the patient by name to check the right patient, explains the procedure to the patient.			
8.	Gives the medicine to the patient one by one with water.			
9.	Records the time, dosage, and medicine on the nurse's notes with signature.			
10.	Keeps all the articles after washing (glasses, containers etc.), dries & replaces.			

Name:

Signature:

Date:

I. Intramuscular injection

1. Definition

It is the introduction of fluid medication in the muscle tissue of the body.

2. Purpose

- To obtain a rapid systematic effect of the drug.
- To administer the drug when it can't be given orally.
- To administer large quantity of solution (2 to 10 ml)
- To prevent the irritation of the lining of the digestive tract by irritating drugs.
- To list the patient's sensitivity to drugs.
- To prevent the drug from being destroyed or rendered ineffectual by the action of digestive juices.

3. General Instructions

- Check the prescription of the physician before administering injection.
- Check for expiry of the drug.
- Read the label and check the strength of the medication.
- Use sharp needle with appropriate size to avoid undue discomfort.
- Keep 2 needles- One for drawing fluid from vial / ampoule & one for administering medication.
- Be sure that patient is not sensitive to drug.
- Administer drug by following 5 "R", - right drug, right route, right patient, right dose & right time.
- Select the right needle for thin & obese patients.
- Don't use any drug which is discolored or has sediments.
- Alternate the site of injection each time to prevent irritation & allowing complete absorption.

4. Sites for injection

- Dorso Gluteal- Upper & outer quadrant of gluteal muscle. } commonly
- Deltoid- three finger below the acromion process } used
- Thighs - Vastus lateralis- lateral aspect of the thigh.
- Ventro gluteal
- Rectus femorsis - Anterior aspect of the thigh.

5. Articles required

- Tray containing
  - Disposable needles with syringes.
  - Sterile dissecting forceps in a jar with antiseptic solution.
  - Spirit swabs in a container.
  - Distill water to dissolve the medication.
  - File to break the ampoule.

- Kidney tray & paper bag.
- Medicine card / kardex / treatment book.

6. Procedure

- Check the medicine order with medication card.
- Explain procedure to the patient.
- Wash hands.
- Check label on vial or ampoule.
- Check again the name, dose, time & mode of injection from the prescription.
- Tap the ampoule to bring drug down ceil over the constricted part with file.
- Clear the rubber cork of vial with spirit and allow to dry before inserting needle.
- Dissolve injection thoroughly.
- Withdraw medicine into the syringe.
- Select the site for injection.
- Change the needle.
- Clear the area with spirit swab.
- Stretch the muscle at 90º angle with quick thrust.
- Withdraw plunger & see that needle is not in the blood vessel.
- Inject medicine slowly.
- Withdraw the needle quickly & massage the area gently.
- Discard the needle after destroying the tip & needle into separate container.
- Make the patient comfortable.
- Chart in the nurse's notes.
- Observe for any reaction for 15 minutes.

7. After care

- Replace all the articles as soon as procedure is over.
- Leave the unit clean & tidy.
- Wash hands after replacing articles.

CHECKLIST FOR INTRAMUSCULAR INJECTION

Patient's Name : _____

I.P. No. :_____

Ward/Unit *:* _____

S. No.	Check List	Yes	No	Remarks
1.	Checks the medicine order with medication card.			
2.	Explains procedure to the patient.			
3.	Washes hands.			
4.	Checks label on vial or ampule.			
5.	Checks again the name, dose, time & mode of injection from the prescription.			
6.	Taps the ampule to bring drug down, cuts over the constricted part with file.			
7.	Cleans the rubber cork of vial with spirit and allows to dry before inserting needle.			
8.	Dissolves injection thoroughly.			
9.	Withdraws medicine into the syringe.			
10.	Selects the site for injection.			
11.	Changes the needle.			
12.	Cleans the area with spirit swab.			
13	Stretches the muscle, at 90º angle inserts needle with quick thrust.			
14.	Withdraws plunger & checks needle is not in the blood vessel.			
15.	Injects medicine slowly.			
16.	Withdraws the needle quickly & massages the area gently.			
17.	Discards the needle after destroying the tip & needle into separate container.			
18.	Makes the patient comfortable.			
19.	Charts in the nurse's notes.			
20.	Observes for any reaction for 15 minutes.			
	AFTER CARE			
1.	Replaces all the articles as soon as procedure is over.			
2.	Leaves the unit clean & tidy.			
3.	Washes hands after replacing articles.			

Name:

Signature:

Date:

J.SUBCUTANEOUS INJECTION

1. Definition

It is an introduction of medicine into subcutaneous connective tissue.

2. Purpose

- To achieve a slow absorption & action rate compared to intramuscular & intravenous injections.
- All general purposes of injection.

3. General Instruction

- Select the appropriate size No. 26 or 26 gauge needle.
- Follow instructions of I.M. injections
- Never force the needle if it is bent.
- Select hypodermic syringe, which has ml & C. C. marking for more accuracy.

4. Sites

- Vascular areas around outer aspects of upper area.
- Anterior aspects of thigh.
- Abdominal tissue.
- Scapular area of upper back.
- Subcutaneous tissue below breast.

5. Articles required

- Same as that of I.M. Injection needles 25 & 26 gauge.
- Hypodermic / insulin syringe.

6. Procedures

- Same as for intramuscular injection.
- Grasp the area surrounding the site of the injection & hold it in a cushion fashion.
- Inject the needle quickly at 45º to 60º angle.
- Release the grasp on the tissue & inject the medication slowly.
- Make the patient comfortable.
- Chart in the nurse's notes.

7. After care of articles

- Replace all the articles after the procedure is over.
- Leave the unit clear & tidy.
- Wash hands after replacing articles.

NURSING PROTOCOL : CHECKLIST FOR SUBCUTANEOUS INJECTION

Patient's Name : _____

I.P. No. : _____

Ward/Unit : _____

S. No.	Check List	Yes	No	Remarks
1.	Checks the medicine order with medication card.			
2.	Explains procedure to the patient.			
3.	Washes hands.			
4.	Checks label on vial or ampoule.			
5.	Checks again the name, dose, time & mode of injection from the prescription.			
6.	Taps the ampoule to bring drug down, cuts over the constricted part with file.			
7.	Cleans the rubber cork of vial with spirit and allows to dry before inserting needle.			
8.	Withdraws medicine into the syringe.			
9.	Selects the site for injection.			
10.	Changes the needle.			
11.	Grasps the area surrounding the site of the injection & holds it in a cushion fashion.			
12.	Injects the needle quickly at 45º to 60º angle.			
13.	Releases the grasp on the tissue & injects the medication slowly.			
14.	Makes the patient comfortable.			
15.	Charts in the nurse's notes.			
	AFTER CARE			
1.	Replaces all the articles after the procedure is over.			
2.	Leaves the unit clear & tidy.			
3.	Washes hands after replacing articles.			

Name:

Signature:

Date:

K.ASSISTING IN INTRAVENOUS INFUSIONS

1. Definition

The introduction of a large amount of fluid and / or electrolytes and other nutrients into the body via veins.

2. Purpose

- To supply fluid & food to the tissues when patient is unable to take orally.
- To restore the volume of blood to normal in haemorrhage, burns, diarrhoea & vomiting etc.
- To dilute poisons & flush the kidneys.
- To prevent & treat shock.
- To alter vascular pressure.
- To supply & meet the patient's basic requirements of calories, water, minerals & vitamins.

3. General Instructions

- Maintain strict aseptic techniques.
- Be sure of solution's type, strength, and amount.
- Avoid entry of air.
- Clamp before the whole amount of fluid finishes.
- Check the apparatus for working condition
- Observe the site for swelling (tissue infiltration leaking & bleeding)
- Observe the patient for unfavourable symptoms.
- Regulate the flow of fluid.
    - Drop/mt= No. of bottles X amount in 1 bottle X drops in 1ml.
      Total time in hours X 60
- Flow and the amount depend upon condition & need of patient, disease nature of fluid.
- Fluid should be at room temperature.
- Ensure I.V. set is changed after 12-24 hrs.

4. Veins Frequently Used

- Veins of the cubital fossa
    - Median Cephalic Vein
    - Basilic Vein
- Ante brachial
- Venous network on back of the hand
- Dorsalis pedis
- Saphenous veins
- Scalp vein (infants)
- Jugular vein.

5. Articles Required

- A tray containing
  - I.V solution
  - Sterile I.V. tubing with drip chamber & clamp.
  - Sterile butterfly / vein flow
  - Sterile syringes
  - Sterile cotton swabs
  - Sterile dissecting forceps in a sterile bottle
  - Gauze pieces.
  - Spirit
  - Makintosh with towel
  - Tourniquet
  - Kidney tray & paper bag
  - Adhesive plaster & scissors
  - Spirit

- I/V stand

6. Procedure

- Explain the procedure to the patient
- Wash hands. Remove the bottle seal from top, clean the top with spirit swab, holding the bottle upright, insert the drip set & air vent into the bottle.
- Close the camp & hang the bottle on the I/V stand about 18'.25" high.
- Connect the needle to the IV tubing open the clamp & flush the IV fluid through the tubing & needle into kidney tray until air is expelled. Clamp the tubing again, apply protective, cap over the needle.
- Prepare few strips of adhesive tapes
- Site preparation – apply a tourniquet firmly 6 to 8" proximal to the site.
- Encourage the patient to clench & unclench the fist rapidly lightly tap the vein with finger tip.
- Clean the area with a spirit swab.
- Physician inserts needle into the vein at 15º to 30º angle & once it enters the vein, makes it parallel with the skin & follow the cause of the vein.
- When back flow of blood occurs into the needle & tubing physician inserts the needle further up into the vein about ¾ to 1".
- Release the tourniquet and open the clamp to let fluid flow
- Secure needle & tubing by adhesive tapes / strips.
- Immobilize with split of required.
- Record in nurse's notes & I/O chart.
  - Time of starting infusion
  - Amount & route of flow
  - Type of fluid given
- Change the bottle whenever it finishes with prescribed IV fluid.

7. After Care

- Dispose the IV set & used cotton into the kidney tray & paper bag
- Remove I V Stand tray from patients bedside
- Clean & replace all equipments to in proper place.
- Watch for edema, discolourisation or haematoma formation.
- Hand over to the shift nurse.

Nursing Protocol : Checklist for assisting in intravenous infusions

Patient's Name_____

I.P. No._____

Ward_____

S. No.	Check List	Yes	No	Remarks
1.	Washes hands. Removes the bottle seal from top, cleans the top with spirit swab, holding the bottle upright, inserts the drip set & air vent into the bottle.			
2.	Closes the camp & hangs the bottle on the I/V stand about 18'- 25" high.			
3.	Connects the needle to the IV tubing, opens the clamp & flushes the IV fluid through the tubing & needle into kidney tray until air is expelled. Clamps the tubing again, applies protective, cap over the needle.			
4.	Prepares few strips of adhesive tapes.			
5.	Site preparation – applies a tourniquet firmly 6 to 8" proximal to the site.			
6.	Encourages the patient to clench & unclench the fist rapidly, lightly taps the vein with fingertip.			
7.	Cleans the area with a spirit swab.			
8.	Physician inserts needle into the vein at 15º to 30º angle & once it enters the vein, makes it parallel			

	with the skin & follow the course of the vein.			
9.	When back flow of blood occurs into the needle & tubing physician inserts the needle further up into the vein about ¾ to 1.			
10.	Releases the tourniquet and opens the clamp to let fluid flow.			
11.	Secures needle & tubing by adhesive tapes / strips.			
12.	Immobilizes with split if required.			
13	Records in nurse's notes & I/O chart. - Time of starting infusion. - Amount & rate of flow. - Type of fluid given.			
14.	Changes the bottle whenever it finishes with prescribed IV fluid.			
	AFTER CARE			
1.	Disposes the IV set & used cotton into the kidney tray & paper bag.			
2.	Removes I V Stands tray from patient's bedside.			
3.	Cleans & replaces all equipments in proper place.			
4.	Watches for edema, discolourization or haematoma formation.			
5.	Hands over to the shift nurse.			

Name:

Signature:

Date:

L.STEAM INHALATION

1. Definition

It is the inhalation of moist plain or medicated vapors into the respiratory tract.

2. Purpose

- To relieve inflammation and congestion of mucous membranes of respiratory tract and para nasal sinuses
- To soften thick, tenacious mucous and help its expulsion from the respiratory tract.
- To provide heat & moisture to prevent dryness of mucous membrane of the lung and upper respiratory passage e.g. tracheostomy.
- To aid in absorption of oxygen
- To relieve spastic condition of larynx and bronchi.
- To provide antiseptic action on respiratory tract e.g. by using Tr Benzoin

3. Methods

- Jug method
- Nelson Inhaler (commonly used)
- Steam Tent
- Electrical Steam Inhaler

4. General Instructions

- Always warm the inhaler before filling with boiled water.
- Inhaler should be filled only two third with boiling water to prevent scalding and to get warm air.
- To prevent scalding of the patient, the spout of the inhaler must be placed in opposite side of the patient, so that it may not touch the skin of the patient.
- Cover the inhaler to prevent heat loss.
- Temperature of the water should be maintained between120-160 degree Fahrenheit (54.4-76.7 degree centigrade)
- Patient should not be allowed to go into cold atmosphere for minimum 2 hours after the treatment.
- Patient should be kept in warm and drought should be prevented before, during and after the inhalation.
- A sputum mug should be placed near patient to spit during inhalation.
- The steam may be given for 15 to 20 minutes at a time.
- Fill the inhaler 2/3 as water should remain just below the spout.
- Always explain the procedure to patient.
- Always watch the patient throughout the procedure
- Place the pt in Fowler's position.

5. Nurses Responsibility:

- Check the patient name, bed no. and other identifications.
- Check the patient diagnosis and the general condition of the patient.
- Check the physicians order and assess the pts ability for self care.
- Assess the level of consciousness and the ability to follow the instructions

6. Articles Required

- Nelsons inhaler with a mouthpiece tightly fit to the neck of the inhaler.
- Bowl or basin to hold the inhaler
- Towel big & small.
- Kettle, gauze piece and a cotton swabs
- Kidney tray and a paper bag.
- Sputum mug

7. Procedure

- Warm the inhaler by pouring a little hot water into the inhaler and emptying it.
- Fill up the inhaler with hot water.
- Cover the mouthpiece with a gauze piece and plug the spout with cotton ball.
- Cover the inhaler with towel and take it to the pt in a basin.
- Remove the plug and instruct the patient to place the lips on the mouthpiece and breathe in the vapour & exhale through nose
- Cover the patient's head and face with a towel.

8. Care of Patients and Articles after Procedure

- Remove the inhaler from the stated time and wipe off the perspiration from the face
- Make the pt comfortable and instruct to remain in the bed for 1-2 hrs.
- Take the articles to the utility room. Empty and clean the articles.
- Dry and replace in their proper places and wash hands.
- Record the procedure on the nurse's record with date and time.
- Record the pts response to the procedure.

NURSING PROTOCOL : **CHECKLIST FOR STEAM INHALATION**

Patient's Name : _____

I.P. No. : _____

Ward : _____

S. No.	Check List	Yes	No	Remarks
1.	Measures the capacity of the inhaler with cold water. Measure the capacity when it is filled half to two third full.			
2.	Warms the inhaler by pouring a little hot water into the jug & emptying it.			
3.	Pours the required amount of water. The water should remain just below the spout.			
4.	Places the mouth piece and closes the jug tightly. See that the mouth-piece is in the opposite direction to the spout.			
5.	Covers the mouth piece with a gauze piece and plugs the spout with a cotton ball.			
6.	Covers the jug with a flannel piece or a towel.			
7.	Places the inhaler in the basin and takes it to the bedside without losing time.			
8.	Places the apparatus conveniently in front of the patient with the spout opposite to the patient. Removes the cotton plug and discard it into the paper bag.			
9.	Instruct the patient to place the lips on the mouth piece and breathe in the vapour. After removing the lips from the mouth piece, breath out the air alternatively, he should breathe in the steam through the nostrils.			

	AFTER CARE			
1.	Removes the inhaler from the patient after the stated time. Wipe off the perspiration from the face.			
2.	Removes the cardiac table and adjust the position of the patient in bed. Make him comfortable.			
3.	Instructs him to remain 1-2 hours in bed to prevent draught.			
4.	Takes the articles to the utility room. - Removes the gauze covering the mouth piece and clean the mouth piece thoroughly. Boil it to prevent cross infection. - All the other articles are cleaned with warm soapy water and then with clean water. Dries & replaces them in their proper places. Washes hands.			
5.	Records the procedure on the nurse's record with date and time. Records the patient's response to the procedure.			
6.	Returns to the patient to assess his comfort & to observe any untoward reactions in him. Offer hot drinks if needed.			

Name:

Signature:

Date:

M.RYLE'S TUBE FEEDING

1. **Definition**
   It is an artificial method of giving fluids & nutrients through a tube that has been passed into esophagus & stomach through the nose or mouth, when oral intake is inadequate or impossible.

2. **Indications**
   - When patient is unable to take food by mouth e.g. unconscious, semi-conscious & delirious patient.
   - Patient who refuses food.
   - Patient with dysphagia, fracture jaw, cleft palate, cleft lips, oral surgery, esophageal or threat surgery etc.
   - When patient is too weak to swallow food.
   - When patient is unable to retain food.

3. **Articles required**
   - A tray containing
     - Gauze pieces in a bowl.
     - Paper bag & kidney tray.
     - 10ml syringe (for testing location)
     - 20ml-50ml syringe for feeding the patient.
     - Glass of feed in a bowl of warm water.
     - Ounce glass (to measure the feed)
     - A bowl with water.
     - Mackintosh with towel.
     - Stethoscope.
     - Face towel and hand towel.

4. **General Instructions**
   - Give only after Doctor's orders
   - Explain procedure to patient & gain cooperation.
   - Remove dentures if any.
   - Give feed at interval of 2, 3 or 4 hrs.
   - Don't exceed 150-300ml / feed.
   - Record intake & output accurately.
   - Watch for complications e.g. nausea, vomiting, diarrhoea, breathing problem etc.
   - Don't force the feed.
   - Prevent air entry inter Ryle's tube while feeding.
   - Keep suction apparatus ready & handy

5. Procedure

- Explain procedure to patient.
- Bring all articles at bedside
- Give fowler's position or raise head with extra pillows.
- Place mackintosh & towel under patient's head.
- Place hand towel under patient's chin to protect linen & garments.
- Remove dentures if any.
- Wash hands.
- Check the tube for placement in stomach by as pirating gastric contents.
- Measure the feed in ounce glass, the feed can be given by syringe method by gravity: Give feed through barrel of syringe while giving the feed don't let the air enter the tube before the air can enter either pour more feed or pinch the tube & raise the barrel, above shoulder level of patient.
- Give some amount of water before & after giving the feed to rinse the tube for rumoring fats or deposits to prevent blockage of tube.
- Then clamp the tube finally to prevent the leakage of feed.
- Clean the face with face towel.
- Remove mackintosh & towel.
- Make the patient comfortable in bed.
- Discard the waste & clean articles with soap & water.
- Dry and replace articles.
- Wash hands & record the procedure:
    - in nurse's notes / nurse's record:
        - Record the time & date.
        - Record amount & type of feed.
        - The reaction of patient after feed, if any.
    - In intake & output chart:
        - Date & time
        - Amount & type of feed.

NURSING PROTOCOL : CHECKLIST FOR RYLE'S TUBE FEEDING

Patient's Name : _____

I.P. No. : _____

Ward : _____

S. No.	Check List	Yes	No	Remarks
1.	Explains procedure to patient.			
2.	Collects all articles at bedside of patient.			
3.	Gives fowler's position or raises head with extra pillows.			
4.	Places mackintosh & towel under patient's head.			
5.	Places hand towel under patient's chin to protect linen & garments.			
6.	Removes dentures if any.			
7.	Washes hands.			
8.	Checks the tube for placement in stomach by aspirating gastric contents.			
9.	Measures the feed in ounce glass, the feed can be given by syringe method by gravity: Gives feed through barrel of syringe while giving the feed does not let the air to enter the tube, before the air can enter either pours more feed or pinches the tube & raises the barrel, above shoulder level of patient.			
10.	Gives some amount of water before & after giving the feed to rinse the tube for remaining fats or deposits to prevent blockage of tube.			
11.	Then clamps the tube finally to prevent the leakage of feed.			
12.	Cleans the face with face towel.			
13	Removes mackintosh & towel.			
14.	Makes the patient comfortable in bed.			
15.	Discards the waste & cleans articles			

	with soap & water.			
16.	Dries and replace articles.			
17.	Washes hands & record the procedure: • In nurse's notes / nurse's record: - Records the time & date. - Records amount & type of feed. - The reaction of patient after feed, if any. • In intake & output chart: - Date & time. - Amount & type of feed.			

Name:

Signature:

Date:

N.OXYGEN ADMINISTRATION BY NASAL CANNULA

1. **Definition**
   It is the administration of oxygen to the patient to prevent or relive Hypoxia or Hypoxemia. (Hypoxia is a condition in which sufficient amount of O2 is not available to meet the metabolic needs of the tissues. Hypoxemia is deficiency of O2 in the arterial blood).

2. **Indications**
   - Breathlessness
   - Cyanosis.
   - Atlectasis
   - Thoracoplasty
   - Poisoning – cyanide poisoning
   - Haemorrhage
   - Shock
   - Critically ill patients

3. **Articles required**
   - Oxygen cylinder or central supply with flow meter
   - Oxygen connecting tube
   - Humidifier with sterile water
   - Tray containing:
     - Nasal cannula and tubing
     - Nasal catheter / face mask as needed
     - Container of sterile water to check the flow

- Torch
- Lubricant Jelly
- Gauze pieces
- Adhesive tape
- Kidney tray
- Paper bag
- Small mackintosh
- Towel
- Swab sticks
- Normal saline

4. Methods of Administering O2 Therapy

- Nasal Catheter
- Nasal Cannula (Prongs)
- Mask Method
- Tent Method

5. General Instructions

- Check doctor's order
- Review safety measures (No leaking of O2, No smoking zone)
- Keep O2 cylinder in secure upright position
- Check that cylinder is full with O2 with all its attachments
- Adjust the flow as prescribed
- All articles collected at bed side
- Clean the nostrils and prongs before administering
- Never discontinue O2 abruptly but gradually by reducing dosage, watch for any signs of toxicity.
- Assess patient's condition
- Never leave the patient alone, leave a calling bell
- Always label the O2 cylinder filled or empty.
- Check for fire precautions as O2 supports combustion.

6. Procedure

(Administering oxygen by nasal cannula/ prongs)

- Explain the procedure to the patient
- Bring all the articles to the bed side
- Assess the condition of the patient
- Review safety precautions; make him comfortable in prop up position or lying down if very sick.
- Wash your hands.
- Connect the nasal cannula to the oxygen setup with humidification. Adjust the flow rate as ordered by doctor. (2-6 litres per minute usually)
- Check that oxygen is flowing out of prongs.
- Clean the nostrils with normal saline swab sticks

- Place the prongs in the patient's nostril after lubrication with jelly.
- Adjust the headband or plastic slide until cannula fits comfortably.
- Use gauze pads beneath the tubing as necessary.
- Encourage patient to breath through nose with mouth closed.
- Assess the patient for response of the therapy.
- Do the charting.

7. After care of the patient and articles

- Keep the patient warm and comfortable.
- Evaluate the patient's progress by observing the vital signs and colour
- Change the nasal catheters at every 8 hours or more frequently, because the mucus may plug the opening of the catheter and block the oxygen.
- When the oxygen is to be discontinued, do it gradually.
- Watch the patient for any untoward symptoms like dysponea, cyanosis etc.

8. Complications/ Hazards

- Infection
- Combustion
- Oxygen toxicity
- Oxygen induced apnoea.
- Retrolental fibroplasia.
- Asphyxia.

NURSING PROTOCOL: CHECKLIST FOR OXYGEN ADMINISTRATION BY NASAL CANNULA

Patient's Name : _____

I.P. No. : _____

Ward : _____

S. No.	Check List	Yes	No	Remarks
1.	Washes hands			
2.	Setup the trolley /tray			
3	Check the orders for dose of oxygen, rate of flow, methods of administering.			
4	Check the tubing for any kinks and			

	obstruction			
5	Explain the procedure to the patients and assess his condition			
6	Give Comfortable positions			
7.	Clean the nostril and Lubricate the prongs.			
8	Administer oxygen by means of cannula (Prongs)			
9	Take the vitals of patient and observe the response of oxygen therapy.			
10	Records the date, time, and effect of the O2 therapy			
11	Replace all the articles after washing drying.			

Name:

Signature:

Date:

O. TRACHEOSTOMY SUCTIONING

1. **Definition**
   Suctioning is removal of secretions from the tracheo-bronchial tree by vacuum (suction) by insertion of a sterile catheter into the tracheostomy opening (artificial airway).

2. **Factors affecting suctioning**
   - Type of suction catheter
   - Source of vacuum pressure
   - Hydration Liquefy secretion)
   - Need for suctioning

3. **Suction Catheter and Appropriate Size (As per age)**

Age	Size
New born & infant	6-8
2 yrs	8-10
2-8 yrs	10-12
8-12 yrs	13-14
Adults	12-18

4. **Indication of Suction**
   - Change in Respiratory Pattern
   - Vibration of loose secretions felt through the chest wall
   - Rhonchi and loose secretions sound
   - Noisy, Gurgling sound

5. **Purposes of Suction**
   - To maintain patent air way
   - To improve ventilation
   - To prevent the effect and complication of retained secretions
   - To obtain a tracheal aspiration specimen for diagnostic test
   - To stimulate coughing
   - To improve gas exchange and tissue oxygenation

6. **Assessment**
   - Assess the patient need for suctioning by performing visual, auditory, tactile, and auscultatory assessment
   - Assess patient's awareness and ability to co-operate
   - Observe level of consciousness to assess hypoxia
   - Assess Blood Gas analysis/pulse oximeter to check the potential Hypoxaemia and cardiac arrhythmia

7. **Equipment**
   - Suction machine with connecting tubes

- Suction catheter (appropriate size)
- Normal saline in a sterile bowl
- Sterile gauze piece
- Towel
- Two pair of sterile glove
- Emergency equipment (Extra tracheostomy tube, emergency medicine kit, artificial respiratory O2, Ambu bag etc.)

8. Steps of Procedure

- Review Doctor's order
- Explain procedure to the patient
- Collect equipments
- Wash hands
- Auscultate lung to breath sound
- Check suction machine
- Provide O2 to the patient if required
- Cover patient chest wall with towel
- Wear gloves (Dominant hand is kept sterile) while non-dominant hand (left hand) is kept clean
- Fill sterile container with normal saline
- Open the new suction catheter packet with appropriate size.
- Attach sterile catheter to suction tubing with a 'Y' connection
- Encourage patient to take deep breath
- Disconnect O2 source (if used) from the tracheostomy tube with clean hand
- Lubricate catheter with normal saline
- Switch on suction machine gently and quickly. Insert catheter into tracheostomy tube with dominant hand.
- Apply intermittent suction during suctioning, lock the end of 'Y' connection with thumb, rotate and withdraw catheter form tracheostomy tube.
- Suction not more than 10 seconds in one stroke.
- Wipe the catheter with the gauze piece then rinse in normal saline.
- Provide rest to the patient and ask him to take deep breath or attach O2 to tracheostomy tube.
- Repeat suctioning 1-2 times more (if needed) use sterile catheter each time.
- Disconnect catheter from suction tube and discard in disposal container.
- Turn off suction machine.
- Give tracheostomy care.

9. After Care of Patient & Articles

- Provide comfortable positions to the patient.

- Remove gloves and dispose off in a proper container
- Assess the respiratory status and compare with previous data.
- Remove all articles from bedside and take to utility room.
- Clean all the equipments & keep them at proper place.
- Wash hands & document the procedure
- Encourage patient to take fluid for maintaining hydration.
- Observe secretion and note abnormalities.

10. Risk of Suctioning

- Vagal stimulation (Bradycandia, Arrythrmia, Cardiac Arrest)
- Mucosal trauma (Pain dysponea, Haemorrhage, Discomfort)
- Hypoxemia, Dyspnoea, Haemorrhage, Discomfort.

NURSING PROTOCOL: CHECKLIST FOR TRACHEOSTOMY SUCTIONING

Patient's Name : _____

I.P. No. : _____

Ward : _____

S. No.	Check List	Yes	No	Remarks
1	Review Doctor's Order			
2	Explain procedure to the patient			
3	Collect equipments			
4	Wash hands			
5	Auscultate lung to breath sound			
6	Check suction machine			
7	Provide O2 to the patient if required			
8	Cover patient chest wall with towel			
9	Wear gloves (dominant hand is kept sterile) while non-dominant hand (left hand) is kept clean			
10	Fill sterile container with normal saline			
11	Open the new suction catheter packet with appropriate size			
12	Attach sterile catheter to suction tubing with a 'Y' connection			
13	Encourage patient to take deep breath			
14	Disconnect O2 source (if used) from the tracheostomy tube with clean hand			
15	Lubricate catheter with normal saline			
16	Switch on suction machine gently and quickly. Insert catheter into tracheostomy tube with dominant hand.			
17	Apply intermittent suction during suctioning, lock the end of 'Y' connection with thumb, rotate and withdraw catheter from tracheostomy tube.			

18	Suction not more than 10 seconds in one stroke			
19	Wipe the catheter with the gauze piece then rinse in normal saline			
20	Provide rest to the patient and ask him to take deep breath or attach O2 to tracheostomy tube.			
21	Repeat suctioning 1-2 times more (if needed) use sterile catheter each time.			
22	Disconnect catheter from suction tube and discard in disposal container.			
23	Turn off suction machine			
24	Give tracheostomy care			
	After Care of Patient & Articles			
25	Provide comfortable positions to the patient			
26	Remove gloves and dispose off in a proper container			
27	Assess the respiratory status and compare with previous data.			
28	Observe secretion and note abnormalities.			
29	Encourage patient to take fluid for maintaining hydration.			
30	Wash hands & document the procedure			
31	Remove all articles from bedside and take to utility room.			
32	Clean all the equipments & keep them at proper place.			

Name:

Signature:

Date:

P.URINARY CATHETERISATION

1. **Definition**
   It is the insertion of urinary catheter through the urethra into bladder.

2. **Purpose**
   - To relieve distension e.g urinary retention, incontinence.
   - As an aid in diagnosis e.g obtaining sterile sample
   - To empty the urinary bladder before operation on pelvic organs.
   - To install medications into the bladder.
   - To induce voiding
   - Patient's ability to achieve & maintain the desired position.

3. **Preliminary Assessment**
   - Check physicians order
   - Signs of urine retention.
   - Condition of perineum for cleanliness.
   - Kind of the lighting in the room.

4. **Articles Required**
   - Screen
   - Extra Sheets for draping
   - Safety pins to secure the drapes
   - Articles for protecting bed linen & for patient s comfort
     - Mackintosh & draw sheets
     - Bedpan & paddings.
     - Pillows to support legs in unconscious patient.

   - Articles for perineal care
     - Sterile Tray containing:
       - Gloves pair
       - Bowl of wet cotton swabs
       - Bowl of dry cotton swabs
       - Artery forceps& dissecting forceps.
     - Kidney tray

   - Articles for Catheterisation
     - Sterile Tray containing:
       - Sterile sheet
       - Gloves
     - Lubricant with applicator/swab/ gauze pieces
     - Foley's Catheter 14-18 French
     - 20ml disposable syringe
     - Sticking plaster

- Kidney tray & paper bag

5. **Steps of procedure**
   - Wash your hands, collect the articles & carry it to bedside
   - Explain the purposes & procedure to the patient.
   - Provide proper light, fold the top linen to foot end and drape the patient with extra sheets& provide privacy with the help of screen
   - Place the patient in dorsal recumbent position with knees flexed and feet about 2ft apart.
   - Remove all the clear articles from bed.
   - Fix urobag to the frame of bed.
   - Keep the sterile catheterization on bed at foot end.
   - Wash hands thoroughly & expose patient perineum.
   - Place sterile sheet under the patient's buttocks & place the sterile articles.
   - Select the catheter.
   - Give sterile perineal care.
   - Lubricate the catheter tip with jelly for about 3"-4" in females & 7"-8" in male.
   - Insert catheter gently into the urethra in rotatory motion until the urine flows into kidney tray after separating labia minora.
   - If specimen is to be taken, hold the end of catheter of over sample bottle.
   - Secure the catheter by inflating bulb with 8-10ml of N.S. or distilled water.
   - Attach catheter to urobag.
   - Place all the articles on tray & cover it.
   - Make the patient comfortable.
   - Measure & chart the amount & character of urine & time of treatment on nurse's notes & maintain intake output chart.

6. **After care of equipment**
   - Take equipments to the utility room.
   - Check & label the specimen & send it for investigation to the laboratory.
   - Measure & discard the urine & paper bag.
   - Wash the equipments with soap water.
   - Reset the tray & complete it & send it to CSSD for sterilization.

Nursing Protocol: Checklist for urinary catheterization

Patient's Name : _____

I.P. No- : _____

Ward : _____

S. No.	Check List	Yes	No	Remarks
1.	Washes hands, collect the articles & carry it to bedside			
2.	Explains the procedure and its purposes.			
3.	Provide good light and privacy.			
4.	Protect bed clothing from soiling.			
5.	Places the patient in dorsal recumbent position with knees flexed and feet about 2 feet apart.			
6.	Wash hands, open sterile tray, wear gloves.			
7.	Clean perineum under aseptic technique.			
8.	Lubricate the catheter before inserting gently in rotatry manner.			
9.	Collect the specimen according to instructions.			
10.	Secure the catheter by inflating the bulb with normal saline / distilled water according to manufacturer recommendation.			
11.	Attach the catheter to the drainage system and fix it to the bed frame.			
12.	Remove the article and make the			

	patient comfortable.			
13	Measure the amount of urine and observe its characteristics and discard it.			
	AFTER CARE			
1.	Take equipments to the utility room. Wash and replace them.			
2.	Check & label the specimen & send it for investigation to the laboratory.			
3.	Wash hands and document the procedure and maintain the output chart.			

Name:

Signature:

Date:

Q. SURGICAL DRESSING

1. **Definition**
   Dressing is a protective covering applied to the wound.

2. **Purpose**
   - To control bleeding
   - To prevent infection.
   - To prevent tissue damage.
   - To promote healing
   - To absorb inflammatory exudates and to promote drainage.
   - To convert the contaminated wound into a clean wound.
   - To prevent skin excoriation.
   - To apply the medication.

3. **General Instruction**
   - Practice strict aseptic technique.
   - Wash hands thoroughly before & after the procedure.
   - Instruments used for one dressing cannot be used for another until they have been re-sterilized.
   - Use mask sterile gloves & gowns for large dressings to minimize the wound contamination.
   - Dressing is not changed at least for 15 minutes after the room has been swept or cleaned.
   - Create a sterile field around the wound by spreading sterile towels.
   - Avoid talking, coughing & sneezing when the wound is opened.
   - Cleaning of the wound should be done from the cleanest area to the less clean area.
   - Always place a dressing directly on to a wound never slide it on from the side.
   - When drains are in place, anticipate drainage & re-enforce the dressing accordingly. The dressing over the drain should not be combined with the dressing on the wound line.
   - The amount of discharge from the wound should be accurately measured.
   - Before doing the dressing, inspect the wound for any complications such as dehiscence & evisceration. If present, report it immediately to the surgeon & immediate steps are to be taken.

4. **Assessment**
   - Check the diagnosis & the general condition of the patient.
   - Check the purpose for which the dressing is to be done.
   - Check the condition of the wound the type of wound, the type of suturing applied.

- Check the physician's order for the type of dressing to be applied & specific instructions.
- Check the patient's name, bed number & other notifications.
- Check the nurse's record to find out the general condition of the wound.
- Check the consciousness of the patient & the ability to follow instructions.
- Check the articles available in the unit.

5. Articles required

- A sterile tray containing:
  - Artery forceps – 1
  - Dissecting forceps – 2
  - Scissors – 1
  - Sinus forceps – 1
  - Probe – 1
  - Small bowl – 1
  - Safety pin – 1
  - Gloves, marks, gown.
  - Cotton balls, gauze pieces, cotton pad etc as necessary
  - Slit dressing towel.

- Unsterile tray containing:
  - Cleaning solution as necessary.
  - Ointments and powders as ordered.
  - Vaseline gauze in sterile containers.
  - Ribbon gauze in sterile container.
  - Swabs sticks in sterile container.
  - Transfer forceps in a sterile container.
  - Bandages, binders, pins adhesive plaster and scissors.
  - A larger bowl with disinfectant solution.
  - Kidney tray and 6-paper bag.
  - Mackintosh & towel

6. Preparation of the patient and environment

- Identify the patient & explain the procedure.
- Provide privacy with curtains & drapes.
- Apply restraints in case of children. As far as possible avoid meal timings.
- Offer bedpan or urinal prior to the dressing.
- Give some analgesics if the patient is in pain.
- Shave the area if necessary to remove the hair.
- Place the patient in a comfortable & relaxed position.
- Close doors & windows to prevent draughts. Put off fans.

- Call for assistance if necessary e.g. to do the unsterile procedures, to transfer sterile supplies etc.
- Protect the bed with a mackintosh and towel.
- Turn the head of the patient to one side, so that the patient may not see the wound & get worried about it.

7. Procedure

Tie the mask.

- Wash hands thoroughly.
- Put on gown, gloves etc. as necessary.
- Open the sterile tray spread the sterile towel around the wound.
- Pick up a dissecting forceps and remove the dressings and put it in the paper bag. Discard the dissecting forceps in the bowl of lotion.
- Note the type and the amount of drainage present.
- Ask the assistant to pour small amount of cleansing solution into the bowl.
- Clean the wound from the centre to periphery discarding the used swabs after each stroke.
- After through cleaning of the wound with dry swabs using the same precautions. Discard the forceps in the bowl of the lotion.
- Apply medications if ordered.
- Apply the sterile dressings. Apply the gauze pieces first & than the cotton pads. Reinforce the dressing on the dependant parts where the drainage may collect.
- Remove the gloves and discard it into the bowl with lotion.
- Secure the dressing with bandage or adhesive tapes.

  N.B.: Removal of the drains or sutures should be done after the cleaning of the wound area.

8. Aftercare of the patient and articles

- Help the patient to dress up and to take a comfortable position in the bed. Change the garment if soiled with drainage.
- Replace the bed linen.
- Remove the mackintosh and towel.
- Take all articles to the utility room. Discard the soiled dressings into a covered container and send for incineration. Remove the instruments and other articles from the disinfectant solution & clean them thoroughly. Dry them. Reset the tray & send for autoclaving. Replace all articles to their proper places. Send the soiled liner to the laundry bag for washing.
- Wash hands.
- Record the procedure on the nurse's record with date & time. Record the condition of the wound, the type and amount of drainage condition of the sutures etc. Report to the surgeon any abnormalities found.

- Return to the bedside to assess the comfort of the patient. Special instructions in the case of the wound are to be communicated to the patient.
- Tidy up the bed & the unit of the patient.

NURSING PROTOCOL : CHECKLIST FOR SURGICAL DRESSING

Patient's Name_____

I.P. No _____

Ward_____

S. No.	Check List	Yes	No	Remarks
1	Check the orders for dressing.			
2	Wash hands			
3	Collect the articles- sterile and clean and take it to the bedside.			
4	Explain the procedure & assess the condition of the patient.			
5	Provide privacy & give comfortable position.			
6	Protect the bed linen with mackintosh and towel.			
7	Wash hands, wear gloves and assess the condition of the wound after removing the previous dressing.			
8	Discarded the dissecting forceps after removing the dressing in an antiseptic lotion			
9	Clean the wound from centre to periphery in each stroke.			
10	Secure the wound with adequate sterile dressing and make the patient comfortable.			
	After Care of patient and Articles			
1	Assist the patient to dress up & change the linen if soiled.			
2	Take all the articles to the utility room and discard the soiled dressing in to yellow container.			
3	Replace all the articles after cleaning and drying.			
4	Wash hands and document the procedure with date, time, type of dressing and amount of drainage etc.			
5	Assess the comfort of the patient again.			

Name:

Signature: Date:

NURSING PROTOCOL :FOR CASUALTY / EMERGENCY

Patient's Name_____

I.P. No._____

Ward_____

S. No.	Check List	Yes	No	Remarks
1.	**Reception of the patient:** • Received by Nursing aids • Patient is wheeled to Emergency Room • Identifies the patient by name • Initial assessment of the condition of patient. • Patient seen by CMO • Carries out the immediate treatment as ordered. • Gives the prescription slip to relatives for • Replacement of drugs.			
2.	**If the patient is critical:** • Wheels the patient to Resuscitation Room • Assists doctor in resuscitation of the patient. • Carries out the treatment as ordered and records. • Observes and monitors the condition of the patient, • Records the vitals. • Coordinates with other support services e.g. Lab./ • Radiology etc. for investigations. • Follows institutional policies regarding legal issues.			

	• Decision for Admission by CMO • Nurse informs to Registration Deptt. telephonically. • Room allotment is done by Registration deptt. • Nurse informs the ICU regarding admission of patient. • Transfers the patient to ICU. • Nurse accompanies the patient along with the • Records and hands over the patient to ICU Nurse (At times doctor also goes along with the patient). • Enters all details regarding patient in the emergency register. • Nurse cleans and takes care of the resuscitation equipment and keeps ready for next use.			
3.	**If a patient is MLC:** • CMO informs the police • Nurse enters MLC on the patient's file.			
4.	**If patient not critical and is to be admitted:** • Carries out the treatment as ordered and records. • Observes and monitors the condition of the patient. • Records vital Temp., Pulse, Respiration, and B.P. • Follows institutional policies regarding legal issues. • Informs Registration Deptt. telephonically. • Gives Admission Request to relative to take to • Registration Desk for Admission formalities. • Sends the patient to the allotted room/ward along with ward attendant.			

5.	**Patient who is not admitted:** • Assesses the condition of the patient. • Carries out the treatment as ordered and records. • Patient may be kept for observation. • Observes and monitors the condition of the patient and record the vitals. • Gives necessary instructions for compliance of treatment and follow up at the time of release.			

Name:

Signature:

Date:

NURSING PROTOCOL :CHECKLIST FOR PRE-OPERATIVE PATIENT

Patient's Name_____

I.P. No._____

Ward_____

Nursing staff are requested to complete the following checklist before sending the patient to operation theatre for any surgery.

S.No.	Check List	Yes	No	Remarks
1.	Consent taken			
2.	PAC done			
3.	Part preparation done			
4.	Prepared part checked by Nursing Supervisor/ Senior Staff			
5.	All investigation report including X-ray, CT Scan, MRI etc. are attached with file.			
6.	Blood grouping & arrangement of blood done			
7.	Patient instructed on deep breathing, coughing and post-operative exercises.			
8.	Patient fasting			
9.	Jewelry, Bangles etc. removed			
10.	Prosthesis removed like dentures (false teeth) hearing aid, eye-glasses, contact lenses etc.			
11.	Nail Polish make up & hair pins removed			
12.	Hair combed and tied			

13.	Voided/catheterized			
14.	Enema given with result			
15.	Thorough bath & patient dressed in hospital clothes			
16.	Information about drug allergy recorded on case file			
17.	Pre-medication given on call from O.T			
18.	Vital signs checked & recorded			
19.	Identification tag tied on patient			
20.	Whether patient is Hbs Ag / HIV +ve			
21.	Naso-gastric tube inserted if ordered			
22.	Patient accompanied and handed over to O.T nurse with case file			

Name:

Signature:

Date:

NURSING PROTOCOL : CHECKLIST FOR POST-OPERATIVE PATIENT

Patient's Name_____

I.P. No._____

Ward_____

S.No.	Check List	Yes	No	Remarks
1	Receive the patient with smile and concern			
2	Identify the patient by asking name & checking with file			
3	Transfer the patient safely from trolley to the post-operative bed			
4	Cover the patient with sheet/blanket			
5	Give comfortable position			
6	Connect oxygen if ordered			
7	Check the drainage for any bleeding, blockage etc. & report accordingly.			
8	Check the I/V line to ascertain that it is functioning as per the doctors order			
9	Check the vital signs, record the same & report if needed			
10	Check the drainage tubes and connect it properly			
11	Observe the wound for any bleeding			
12	Maintain silence and send the relatives out			
13	Administer antibiotics or any other drugs as per order			

14	Continue recording vital signs 4 hrly / as necessary as per the patient's condition			
15	Maintain intake and out put chart			
16	Prepare, send the diet slip & ensure that the patient take the diet as per the order			
17	Give steam inhalation thrice in day			
18	Aspirate as indicated , record and report if needed			
19	Give sponge bath daily till the patient is able to take by himself			
20	Take care of mouth, pressure points & personal hygiene			
21	Encourage early ambulation of patient			
22	Encourage deep breathing and coughing exercises			
23	Give moral support to patient & family			
24	Maintain records legibly and correctly			
25	Report / call the doctor when required			

Name:

Signature: Date:

R.CARDIO PULMONARY RESUSCITATION

1. **Definition**

CPR is a means of artificial supporting circulation and oxygenation. It seeks to restore and maintain the patient's respiration and circulation. It is a systematic approach to life support.

2. **Purpose**

- To restore cardio pulmonary functioning.
- To prevent irreversible brain damage from anoxia.

3. **Assessment:**

- Determine the client is unconscious. Shake the patient & shout to confirm unconsciousness.
- Assess for presence of respirations.
- Assess carotid artery for pulse.

4. **Equipment:**

- A hard surface: patient may be placed on floor, ground or wooden board.
- Resuscitation masks or face shield (optional).

5. **ABC of Basic life support:**

The steps are:
- Airways,
- Breathing
- Circulation.

6. **Procedure - One rescuer and One client**

- Ensure safety of rescuer & victim.
- Check the victim & see if he responds: Gently shake his shoulders & ask loudly "Are you all right?"
    1. <u>If he responds by answering or moving:</u>
        - Leave him in the proper position.
        - Send for help.
        - Reassess him regularly.
    2. <u>If he does not respond:</u>
        - Shout for help.
        - Turn patient on to his back while supporting head and neck. Place a board under the back or place client on the floor.
        - Open the airway.
        - Use a head tilt – chin lift method.

- Place your hand on client's forehead and gently tilt this head back keeping your thumb and index finger free to close its nose if rescue breathing is required.
- Remove in a visible obstruction from the victim's mouth, including dislodged dentures.
- With your fingertips under the point of victim's chin, lift the chin to open the airway.
- Use the modified jaw thrust maneuver, if a neck injury is suspected.

- Keeping the airway open, look, listen and feel for normal breathing:
  - Look- for chest moment.
  - Listen- At the victim's mouth for breath sounds.
  - Feel – for air on your Chest.
  - Look, listen and feel for not more than 10 seconds to determine if the victim is breathing normally.

3. <u>If he is breathing normally</u>
   - Turn him into the recovery position
   - Send for help.
   - Check for continued breathing.
4. <u>If he is not breathing or is only operational gasps or is weak attempts on breathing:</u>
   - Send for help.
   - Turn the victims onto his back
   - Give two slow effective rescue breaths each of which makes the chest rise and fall.
   - Pinch the client's nostrils with thumb and index finger of hand holding the forehead.
   - Ensure head tilt & chin lift.
   - Take a deep breath to fill your lungs with O2 and place your mouth around the patient's mouth with a tight seal.
   - Blow steadily into his mouth, take two seconds to deliver, watch his chest rise.
   - Take another breath & repeat sequence.
   - Ventilate 2 full breaths.

5. <u>Check the client for sign of circulation:</u>
   - Check the carotid pulse for 5-10 seconds on the side next to which you are kneeling.
6. <u>If pulse is present continue to deliver breaths at the rate of 10-12 per minutes or a breath every 5-6 seconds.</u>
7. <u>If client is pulse less, start chest compression</u>
   - Locate the lower half the sternum, with your hand that is nearest to the victim's feet.
   - Place the heel of one hand over the lower half of the sternum, avoiding the xyphoid process.

- Place the heel of the other hand on the top of the first.
- Interlock the fingers of both hands & keep them up and off of the chest wall.
- Position yourself vertically above the client's chest with your arms straight.
- Compress the sternum 1.5" – 2"; then completely release compression while maintaining correct hand position. Repeat in a smooth rhythm and release sequence 15 times at a rate of 100 compressions per minute.
- Combined rescue breathing compressions, after 15 compressions tilt the head, lift the chin and give two effective breaths.
- Repeat 4 cycles of 15 chest compressions and 2 ventilations.
- Reassess for carotid pulse. Continue CPR.

7. Documentation:

Document – why you initiated it?

- Whether the victims support from cardiac or respiratory arrest?
- When you found the victims & started CPR?
- How long the victim received CPR?
- Note response and any complications; Intervention to correct complications.

8. Complications:

- Sternal and rib fractures.
- Pneumothorax, haemothorax
- Injury to the heart and great vessels.
- Organ laceration – liver & spleen.
- Aspiration of stomach contents.

NURSING PROTOCOL XVIIA: CHECKLIST FOR CARDIO PULMONARY RESUSCITATION

Patient's Name : _____

I.P. No. :_____

Ward : _____

S. No.	Check List	Yes	No	Remarks
1.	Assess the condition of the patient to ascertain the need for CPR			
2.	Assess the responsiveness by shaking and calling the patient.			
3.	Assess the cardiac and respiratory status of the patient (Presence of respiration and pulse) and previous history of cardiac arrest.			
4.	Check that CPR kit is complete			
5.	Follow the steps of ABC of basic life support.			
6	Ensures the safety of self and the victim.			
7	Place the patient on hard surface in supine position and rescuer also in correct position.			
8	Make sure that airway is cleared by proper position (Hyperextension of head & neck) and artificial dentures are removed.			
9	Initiate mouth-to-mouth breathing if breathing not restored.			
10	Ensure the closing of nostrils of victim with thumb and index finger and enclosing his mouth with rescuers mouth to maintain the air tight seal for effective ventilation of lungs.			
11	Repeat the procedure 12-20 times at the rate of one inflation every 3-5 sec.			
12	Ensure the inflation of lungs corresponds to the respiration of the victim.			
13	If victim is pulse less, give cardiac compression following initial four rapid breaths to maintain circulation.			
14	Correct location of lower half of the			

	sternum when cardiac compression is used.			
15	Artificial breathing and cardiac massage corresponds to normal respiration and pulse rate 5:1 with two rescuers and 15:2 with one rescuer.(cardiac message : breathing).			
16	Ensure the establishment of respiration and circulation: constriction of pupils, regular pulse, normal B.P, normal skin colour & rhythmic respiration.			
17	Observe for any complications: sternal and rib fracture, pneumothorax.			
18	Document the procedure, date, time, method and response of patient.			
	After care of patient			
1	Make the patient comfortable.			
2	Observe for any complication again and take appropriate action.			

Name:

Signature:

Date: